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Review Article

What Is Known About the Psychodermatology Clinic Model of Care? A Systematic Scoping Review

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Abstract

Background: Psychodermatology focuses on the interaction between skin and mental health. Existing research discusses the recognition and treatment of these disorders. However, little is known about the operational structure of subspecialised psychodermatology clinics.

Objective: To identify literature on the structure and logistics of delivering a psychodermatology service.

Methods: A systematic search of MEDLINE, PsycINFO, Embase, and Google Scholar was performed. Articles were included if they discussed the concept and organisation of a psychodermatology practice.

Results: We identified 693 studies; after screening titles and abstracts, 35 full-text articles were assessed, and 17 were included in the scoping review. Most articles discussed aspects of clinic organisation in general; others discussed management of a clinic in the context of specific diseases or made recommendations on incorporating psychotherapeutic techniques in a solo practitioner setting. A weekly multidisciplinary clinic or resident teaching clinic with joint dermatologist-psychiatrist consultation is the most commonly reported model. Specifically, a stepped level of care approach is often used, where patients in increasing level of distress are stratified to the appropriate team of trained professionals. A corresponding curriculum to supplement practitioners' knowledge is recommended.

Conclusions: Various clinic models have been described to provide specialised psychodermatology care in specific settings. Research is needed to assess the impact of these multidisciplinary models of care on patient outcomes and health care costs.

Keywords

psychodermatology, psychocutaneous disease, clinic management, practice models, multidisciplinary care

Psychodermatology, or psychocutaneous medicine, refers to the interaction between the skin and nervous system. In 30% to 60% of dermatological patients, a psychiatric disorder is found related to their skin disease.[1]

Although a universally standard classification system has not yet been developed, many of the psychodermatologic disorders can be divided into 3 categories: psychophysiologic, primary psychiatric, and secondary psychiatric disorders. Psychophysiologic disorders are conditions that are precipitated or exacerbated by emotional stress, such as psoriasis or eczema. Primary psychiatric disorders have an underlying psychiatric problem with self-induced skin disorders (eg, delusions of parasitosis, trichotillomania). Secondary psychiatric disorders involve emotional distress that develops as a result of having skin disease (eg, vitiligo, alopecia areata). It is important to note that many of these conditions can overlap as psychodermatologic patients often have complex biopsychosocial histories.[2]

Management of these patients has proven to be difficult, as psychodermatologic patients can be recalcitrant. Dermatologists may find their management challenging, time-consuming, and aversive.[3,4] The study of psychodermatology is not new, however, as the UK Psychodermatology Working Party was established to connect health care professionals with this subspecialty interest.[5] In the United States, the Association of Psychoneurocutaneous Medicine of North America (APMNA) was developed as a step towards propagating education and awareness.[6]

With the growing acceptance and research on psychodermatological management, it is an opportune time to examine how these services are delivered. Numerous studies have identified dermatologists' gap in knowledge about psychodermatologic services; however, resources on the integration of these services into practice are lacking.[7-9] Knowledge

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synthesis on the logistics and organisational structures of providing liaison care could thus be used to inform clinic practice models. The objective of this scoping review is to draw together what is known about the models of care for structuring a psychodermatology practice.

Methods

A systematic scoping review of the literature on psychodermatology models of care was conducted. Scoping reviews serve to provide an overview of the volume, array, and nature of research activity through mapping the available literature for a field of study.

Search Strategy

Information specialists assisted with preparing a detailed search strategy for each database, combining clusters of terms focused on models of care for structuring a psychodermatology practice. The MEDLINE, PsycINFO, and Embase databases were searched using the OVID interface, with a limit applied for articles published in English. Google Scholar was another source searched using exact phrase limitation. The search results were exported to Mendeley reference manager for duplicate removal.

Study Selection

Articles were included if they related to the format and organisation of a psychodermatology practice run by a family physician, psychiatrist, dermatologist, or various combinations thereof. Peer-reviewed articles, abstracts, reviews, and working party reports were also included. Articles were first screened by title and then by abstract for relevance before reviewing the full text. Inclusion for final review was based on the consistency of the articles with the theme of psychodermatology clinic practice models. Two reviewers (I.M. and S.Z.) independently screened each title and abstract to exclude articles that were clearly ineligible. Full-text papers of the remaining articles were then reviewed to confirm eligibility. References of included articles were also scanned for further relevant articles. Discrepancies were resolved through discussion and arbitration by a third reviewer (A.-W.C.).

Data Synthesis

A standardised form was used to extract data from the studies, and guiding questions were used to refine extraction. These questions were as follows: "What tools are recommended for intake and assessment of psychodermatologic patients?" "Who is included in the care team?" and "What interventions and methods of delivery are proposed?" The following information was collected:

- General characteristics: lead author, year of publication, country of study
- Study characteristics: type of article and objective
- Logistics of psychodermatology practice: care team, assessment, patient contact time, clinic length of operation, interventions, and challenges (Table 1)

Results

As shown in Figure 1, 693 sources were identified for review from the databases searched. After excluding articles based on review of titles (n = 535) and abstracts (n = 123), we reviewed 35 full-text articles and included 14 in the review. The references of the 14 included articles were scanned and 3 relevant reports were identified that were not identified by our database search. These articles were combined with the 14 results from our systematic review for a total of 17 articles included in this systematic scoping review (Figure 1). Articles were excluded for various reasons, including describing clinical manifestation of psychodermatological diseases (n = 2), not discussing clinic management (n = 4), or insufficient information as they were editorials and conference abstracts (n = 15).

Seventeen articles were identified that discussed the concept and organisation of a psychodermatology clinic. These presented clinics from a global setting, including Europe,[5,10-16] Asia,[17-19] and North America[20-24] and South America.[25]

Of these articles, several focused solely on the logistics of a psychodermatology clinic in general.[5,10,12,18-22,24] Others discussed a psychodermatology delivery model in the context of a specific disease.[11,14,17,23,25] One article focused on incorporating psychodermatology methods into a practice when there is limited access to a multidisciplinary team.[13] Lastly, articles also discussed barriers to operating this clinic format.[15]

Analysis of the articles elucidated common themes of care team structure, consultation and assessment, and management.

Care Team Structure

In some models, patients are seen simultaneously by the dermatologist and psychiatrist in the same room, as recommended for producing the best treatment outcomes.[5,18,20-23] Other models involved separate evaluations by the dermatologist and psychologist[12,19] or outside of the clinic in a non-threatening family room setting within the hospital.[17] For independent practice or settings where access to a psychologist or psychiatrist is unavailable, the dermatologist can obtain training from continuing medical education or partake in a recognised psychotherapy program. In this case, the dermatologist should be comfortable with prescribing antidepressants or collaborate with the general practitioner.[13]

Beyond the dermatologist-psychiatrist dyad, the use of an interdisciplinary team, including nurse specialists, primary

Table 1. Structure and Management of Reported Psychodermatology Clinic Models Included in the Scoping Review.

	Management		Operational Factors	
Clinic Type and Description	Assessment Tools	Interventions	Timing	Challenges
General interdisciplinary clinic[5,10,16-18,22] • Joint assessment by dermatologist, psychiatrist/psychologist, and specialist nurses	DLQI, HADS, Skindex 29, Cardiff Acne Disability Index, MSE, PeDeSI, BDI, BSI	***Nonpharmacologic*:** habit reversal, CBT, interpersonal therapy and relationship counselling, depression and psychosis management, hypnotherapy, psychoeducation, relaxation techniques, art therapy ***Pharmacologic*:** antipsychotics, antidepressants	45-60 min for new patients, 20-30 min follow-up Weekly, twice-weekly operation, bimonthly 2-4 patients/wk	***System factors*:** health care funding systems, lack of formal training, long consultation times, interdisciplinary cooperation ***Patient factors*:** stigma of psychological treatment, long-term follow-up, clinic distance
Specialised clinic[11,14,23] • Disease-specific (eg, psoriasis) clinic with joint assessment by dermatologist and psychiatrist	HADS, PAID, psychiatric symptoms, and mental health history	***Nonpharmacologic*:** CBT, relaxation techniques, supportive psychotherapy, praise and reassurance ***Pharmacologic*:** antidepressants, anxiolytics, psychotropic drugs	30 min/session Weekly 3-11 sessions/wk	***Patient factors*:** patient refusal of psychiatric assessment, long-term follow-up
Teaching clinic[20,21,24] • Joint assessment by dermatologist, psychiatrist/psychologist, residents, and medical students	I-IV classification based on degree of psychological impact of their dermatological disorder	***Nonpharmacologic*:** psychotherapy, hypnosis ***Pharmacologic*:** psychotropic drugs	30-50 min/session Weekly mornings to twice weekly 3-5 patients seen per week	***System factors*:** funding, acceptance of psychologist in dermatology department, clinician countertransference ***Patient factors***: complexity, missed appointments
Pediatric clinic[25] • Children and parents' support groups facilitated by child psychiatrist, dermatologists, and medical students	CDLQI, FDI, McGill Pain Questionnaire	***Nonpharmacologic*:** external support groups providing educational sessions on disease, treatment, and sharing experiences	90 min/session Biweekly	Not reported
Independent assessment clinic[12,13,15,19] • Dermatologist or primary care physician with separate evaluation by psychiatrist or psychologist	Specific psychodiagnostic tests, genogram	***Nonpharmacologic*:** CBT, biofeedback training, psychotherapy, habit reversal, relaxation training, distraction technique, hypnotherapy, psychoeducation ***Pharmacologic*:** psychotropic drugs	>15 min/session or 4-5 patients/2.5 h also reported Weekly	***System factors*:** funding, organisational logistics, lack of formal training, discomfort with psychocutaneous management ***Patient factors*:** noncompliance with psychological interventions

BDI, Beck Depression Index; BSI, Brief Symptom Inventory; CBT, cognitive-behavioural therapy; CDLQI, Children's Dermatology Life Quality Index; DLQI, Dermatology Life Quality Index; FDI, Family Dermatitis Impact; HADS, Hospital Anxiety and Depression Scale; I-IV, in-house developed classification; MSE, Mental State Examination; PAID, Problem Areas in Diabetes Scale; PeDeSI, Person Centered Dermatology Self-Care Index.

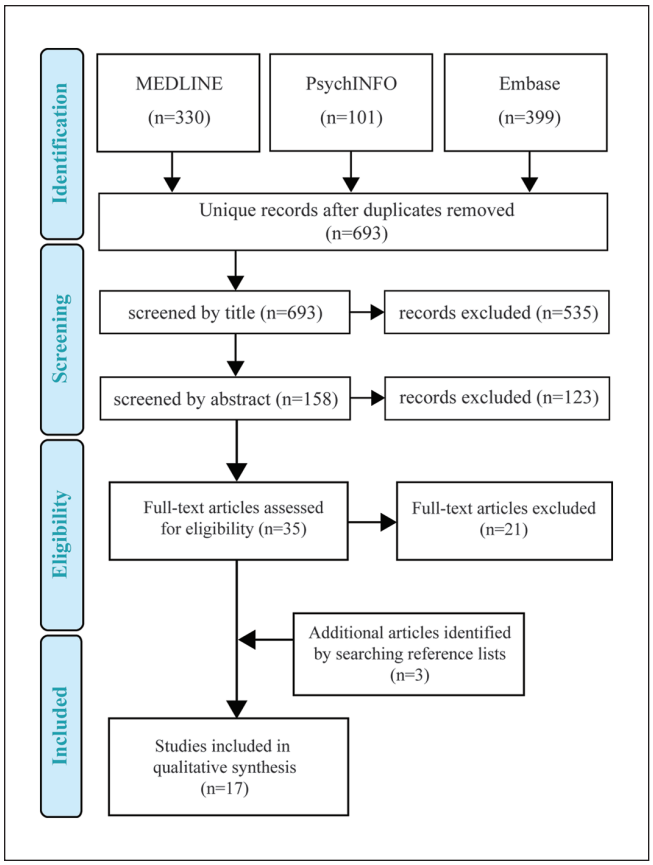

Figure 1. Preferred Reporting Items for Systematic Reviews and Meta-Analysis (PRISMA) flow diagram.

care physicians, social workers, pediatricians, geriatricians, and child and adolescent mental health specialists, can help to serve specific patient populations.[10,11,23] Outside of the clinic, 1 randomised controlled trial (RCT) describing the use of support groups and educational sessions facilitated by a psychiatrist with medical student volunteers has been shown to positively affect patients and their families.[25] Another randomised trial using supportive psychotherapy with diabetic foot syndrome patients also found improvements in depression, anxiety, and diabetes-related problems in the intervention group over the control group (general medical management).[14]

The organisation of a resident-centered teaching clinic has also been described, where the resident consults a psychologist on a weekly basis when they identify a patient with psychological issues. This care model not only exposes residents to the psychological dimension of dermatologic practice but also allows residents to have continuing contact with the psychologist over several years of training.[20,21]

Assessment and Consultation

During the intake appointment, the Dermatology Life Quality Index (DLQI), Hospital Anxiety and Depression Scale (HADS), Beck Depression Index (BDI), and Brief Symptom Inventory (BSI) are commonly used for assessment.[5,18] For pediatric populations, the Children's Dermatology Life Quality Index (CDLQI) is often used.[25]

The time allocated for each patient is generally 45 to 60 minutes for new patients and 20 to 30 minutes for follow-up visits.[5,10,17,21] Many clinics operate once or twice per week with a capacity of 2 to 5 patients per day.[18,19]

Management

Cognitive-behavioural therapy (CBT) and biofeedback were psychotherapeutic tools used in conjunction with dermatologic treatment and pharmacotherapy.[5,11,12,19,26] In milder cases, supportive therapy can be used instead of psychiatric medications.[14,25] Several groups discussed the use of a stepped provision of care approach, in which patients with increasing levels of distress are stratified into 3 groups of management from the lowest to highest level of trained professionals.[16,27] Practitioners with level 1 skills are dermatologists or nurses capable of identifying minor mental health problems. They can assess the patient with a quality-of-life measure such as the DLQI, work with the patient to create a systematic care plan, and offer resources for self-management of medications and psychological factors such as sleep. Those with level 2 skills can be a dermatologist, psychologist, or psychiatrist able to assess those with moderate distress using a mental health assessment to obtain details of their mood or behaviour that can be problematic for self-management. Management can include low-intensity CBT, lifestyle modification, and stress management. Practitioners with level 3 skills (dermatologist, psychologist, or psychiatrist) are able to manage patients with severe distress (eg, body dysmorphic disorder or severe depression). An in-depth mental health assessment is done and management requires training in high-intensity CBT, interpersonal therapy, and specific depression or psychosis treatment.[16] Although there are slight variations in stepped care delivery, Figure 2 shows the general algorithm structure used by these clinics.[5,11]

Few studies addressed the financial sustainability of a psychodermatology clinic. As this clinic format has low patient capacity, 1 article discussed areas contributing to financial difficulty, such as a lack of secretarial services, high rate of missed appointments, and lack of existing cost analysis data.[18] Another recommended the use of psychiatry billing fees rather than general dermatology consultation fees to provide adequate compensation for the longer time spent managing the patient. Although several articles suggested that a psychodermatology clinic model is cost-effective, quantitative data are rarely available.[5,9,10,28,29] The health care utilisation cost per patient was £1210/y in a psychodermatology clinic compared to £8063/y in a general dermatology clinic in the United Kingdom.[5] These savings were due to the avoidance of unnecessary investigations and referrals.[5]

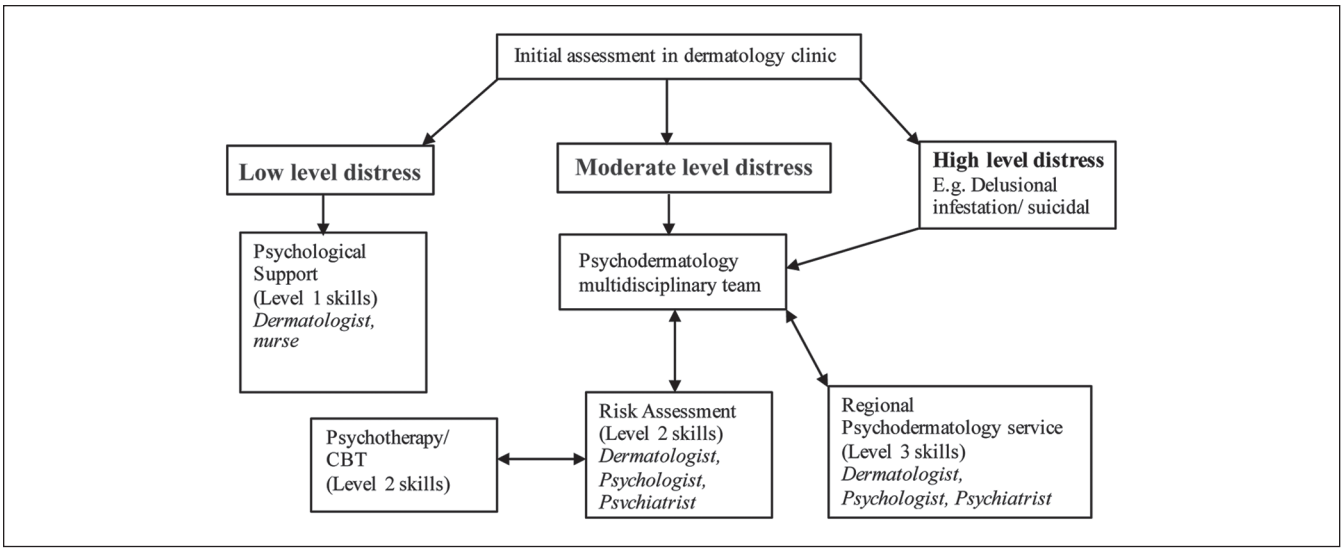

Figure 2. Stepped provision of psychodermatology services. Modified from Bewley et al.[16] CBT, cognitive-behavioural therapy.

In a teaching psychodermatology clinic in the United States, research grants and insurance payments provided sources of funding.[20]

Discussion

We analysed the psychodermatology practice literature to understand 3 areas of clinic structure: care team composition, referrals and consultations, and patient management.

Trends on clinic structure extracted in the review suggest a joint dermatologist-psychiatrist core team, with additional allied health support and primary care doctors (particularly general practitioners with a special interest in dermatology), if available.[30] Practitioners can use standard scales to screen and assess patients' mental health before referral to the psychodermatology clinic, such as the DLQI or HADS. In addition to dermatologic and psychiatric therapies, the stepped-level approach to management was used by several clinics.

Limitations of the Studies

There are few published descriptions of psychodermatology clinics, and all of these articles described experiences in a single clinic. Health care systems in different countries may also limit generalisability of our findings. European psychodermatology groups provided most of the published information on models of delivering care.

Another limitation involved our restriction to English-language articles. Our search also did not include grey literature, theses, or dissertations. Several conference abstracts were excluded because a corresponding research paper or poster could not be found. As per scoping review methodology, the quality of the articles was not evaluated.

Implications

Commonalities amongst the clinic models emphasise areas of need, including practitioners' comfort with the topic, time, and multidisciplinary collaboration in some capacity. Understanding the operational management of a psychodermatology practice is as important as clinical management.

Existing literature calls for a multidisciplinary collaborative approach over a simple referral to a psychiatrist. The format of a consultation-liaison clinic was suggested as the ideal approach for integrated care, consisting of dermatologists, psychologists, and psychiatrists.[12] As psychodermatologic patients require more than an evaluation of the skin manifestations, a thorough evaluation of their social, familial, and occupational issues should also be considered.[7] Treatment often involves CBT, psychotropic drugs, biofeedback training, or psychotherapeutic anxiety management techniques.[31] In 2 RCTs, these psychotherapeutic interventions have been shown to positively influence patients' quality of life and psychological impact of their disease.[14,25] As dermatologists may be uncertain about prescribing anxiolytics, antidepressants, or antipsychotics, the liaison approach allows the patient to receive dermatologic treatment and the appropriate psychiatric management.[8,32] In doing so, this approach helps mitigate the risks of a psychiatric referral, whereby the patient may experience the stigma of being labelled as mentally ill.[1,12]

A substantial gap in the literature is the absence of evaluative criteria and national guidelines on a clinic's effectiveness. In addition to patient outcomes, clinic-focused quality metrics such as cost-benefit analyses, productivity and throughput-based measures, risk analysis, and practice sustainability have not been thoroughly examined. A conference poster abstract from the United Kingdom calculated the cost savings of 4 dermatitis artefacta cases and found dedicated

psychodermatology services cost approximately 14.5% of the total costs of referrals across different specialties. However, firm conclusions could not be inferred from this small study.[29]

The educational model of a psychodermatology clinic is a promising yet little studied approach. For many residency training programs, psychodermatology is not part of the curriculum.[33] As a result, dermatology residents and staff may not have formal training on addressing the mental health needs of these patients.[7] As the prevalence of psychiatric issues has been increasingly recognised in recent decades, this will drive a greater push for collaborative care.[34] The establishment of an internationally recognised training programme or certificate for psychotherapy at the dermatology practice level could be a possible alternative for practitioners aiming to supplement their knowledge.[13]

Bringing the psychodermatology model of care into a Canadian context is a certain possibility. The successful implementation of psychodermatology clinics in various health care systems globally suggests that this care model can be tailored to a Canadian system. In fact, the Skin Health Liaison Clinic in British Columbia uses a psychodermatology model and is currently the 'only clinic of its kind' (http://www.providencehealthcare.org/dermatology-skin). Although this clinic is new, its establishment reflects the increasing acceptance of psychodermatology models in Canada and, with time, sets the precedent for further research in this area.

Conclusion

Psychodermatology is an emerging and important cross-disciplinary field that warrants greater attention in terms of clinical practice and research. Future studies in psychodermatology can build upon well-established interdisciplinary fields, such as psycho-oncology with regards to clinic management and challenges.[35] As more studies of practice models and their impact come to light, we will come to better understand how to best facilitate effective and efficient care for these patients.

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Declaration of Conflicting Interests

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References

1. Basavaraj KH, Navya MA, Rashmi R. Relevance of psychiatry in dermatology: present concepts. *Indian J Psychiatry*. 2010;52(3):270-275.
2. Koo J, Lebwohl A. Psychodermatology: The mind and skin connection. Am Fam Physician. 2001:1873-1878.
3. Koblenzer CS. Psychosomatic concepts in dermatology. *Arch Dermatol*. 1983;119:501-512.
4. Van Moffaert M. Training future dermatologists in psychodermatology. *Gen Hosp Psychiatry*. 1986;8(2):115-118.
5. Aguilar-Duran S, Ahmed A, Taylor R, Bewley A. How to set up a psychodermatology clinic. *Clin Exp Dermatol*. 2014;39(5):577-582.
6. Rivers J. Why psychodermatology is gaining ground. *J Cutan Med Surg*. 2013;17(1):1-2.
7. Jafferany M, Vander Stoep A, Dumitrescu A, Hornung RL. The knowledge, awareness, and practice patterns of dermatologists toward psychocutaneous disorders: results of a survey study. *Int J Dermatol*. 2010;49(7):784-789.
8. Gee SN, Zakhary L, Keuthen N, Kroshinsky D, Kimball AB. A survey assessment of the recognition and treatment of psychocutaneous disorders in the outpatient dermatology setting: how prepared are we? *J Am Acad Dermatol*. 2013;68(1):47-52.
9. Lowry CL, Shah R, Fleming C, Taylor R, Bewley A. A study of service provision in psychocutaneous medicine. *Clin Exp Dermatol*. 2014;39(1):13-18.
10. Marshall C, Taylor R, Bewley A. Psychodermatology in clinical practice: main principles. *Acta Derm Venereol*. 2016;96(217):30-34.
11. Mohandas P, Bewley A, Taylor R. Dermatitis artefacta and artefactual skin disease: the need for a psychodermatology multidisciplinary team to treat a difficult condition. *Br J Dermatol*. 2013;169(3):600-606.
12. Panconesi E. Psychosomatic factors in dermatology: special perspectives for application in clinical practice. *Dermatol Clin*. 2005;23(4):629-633.
13. Poot F, Sampogna F, Onnis L. Basic knowledge in psychodermatology. *J Eur Acad Dermatol Venereol*. 2007;21(2):227-234.
14. Simson U, Nawarotzky U, Friese G, et al. Psychotherapy intervention to reduce depressive symptoms in patients with diabetic foot syndrome. *Diabet Med*. 2008;25(2):206-212.
15. Ocek T, Kani AS, Baş A, et al. Psychodermatology: knowledge, awareness, practicing patterns, and attitudes of dermatologists in Turkey. *Prim Care Companion CNS Disord*. 2015;17(2):10.4088/PCC.14m01628.
16. Bewley A, Affleck A, Bundy C, et al. Working party report on minimum standards for psycho-dermatology services. 2012. http://www.bad.org.uk/shared/get-file.ashx?itemtype=document&id=1622. Accessed October 18, 2016.
17. Chung WL, Ng SS, Koh MJA, Peh LH, Liu T-T. A review of patients managed at a combined psychodermatology clinic: a Singapore experience. *Singapore Med J*. 2012;53(12):789-793.
18. Orion E, Feldman B, Ronni W, et al. A psychodermatology clinic: the concept, the format, and our observations from Israel. *Am J Clin Dermatol*. 2012;13(2):97-101.
19. Shenoi S, Prabhu S, Nirmal B, Petrolwala S. Our experience in a psychodermatology liaison clinic at Manipal, India. *Indian J Dermatol*. 2013;58(1):53.
20. Gould WM. Teaching psychocutaneous medicine. *Arch Dermatol*. 2004;140:282-284.
21. Gould WM, Gragg TM. A dermatology-psychiatry liaison clinic. *J Am Acad Dermatol* 1983;9(1):73-77.

22. Rubin Z, Rabiner CJ, Seides SW. Preparation for organization and operation of a new department. *Arch Dermatol*. 1966;93(4):466.
23. Locala JA. Current concepts in psychodermatology. *Curr Psychiatry Rep*. 2009;202(1):211-218.
24. Silvan M. An integrated approach to the treatment of psychodermatology patients. *Dermatol Psychosom*. 2001;2(2):86-92.
25. Blessmann Weber M, De Tarso Da Luz Fontes Neto P, Prati C, et al. Improvement of pruritus and quality of life of children with atopic dermatitis and their families after joining support groups. *J Eur Acad Dermatology Venereol*. 2008;22(8):992-997.
26. Orion E, Wolf R. Psychological factors in skin diseases: stress and skin: facts and controversies. *Clin Dermatol*. 2013;31(6):707-711.
27. Thompson AR. Treatment challenges: getting psychodermatology into the clinic. Dermatological Nursing. 2014;13(4):26-31.
28. Atkar R, Bewley AP. The cost effectiveness of a dedicated psychodermatology service in managing patients with dermatitis artefacta. *Br J Dermatol*. 2012;167:43.
29. Harper N, Kennedy L, Martin KR GJ. Cost-effectiveness in psychodermatology: a case series. *Br J Dermatol*. 2015;173:201-202.
30. Bewley A, Affleck A, Bundy C, et al. Psychodermatology services guidance: the report of the British Association of Dermatologists' Psychodermatology Working Party. *Br J Dermatol*. 2013;168(6):1149-1150.
31. Wong JW, Koo JY. Psychopharmacological therapies in dermatology. *Dermatol Online J*. 2013;19(5):18169.
32. Situm M, Dediol I, Buljan M, et al. Delusion of parasitosis: case report and current concept of management. *Acta Dermatovenerol Croat*. 2011;19(2):110-116.
33. Clay R. The link between skin and psychology. *Monit Psychol*. 2015;46(2):56.
34. Hidaka BH. Depression as a disease of modernity: explanations for increasing prevalence. *J Affect Disord*. 2012;130(3):205-214.
35. Holland JC. History of psycho-oncology: overcoming attitudinal and conceptual barriers. *Psychosom Med*. 2002;64(2):206-221.

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